

SIC 2027 · INAUGURAL EDITION

THREE-HALL SCIENTIFIC PROGRAM PLAN

HYDERABAD SPORTS INJURIES CONCLAVE

29TH & 30TH JANUARY 2027

Prepared for the Scientific Committee

Researched against IASCON · IASMCN · ISKSAA · ISAKOS · AOSSM · ESSKA · IFSPT · NSCA
and the IOC, WADA and ACSM consensus literature

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WHY THREE HALLS

One hall forces a compromise programme: surgeons sit through nutrition talks, physiotherapists sit through arthroscopic technique. Three parallel halls give each profession two full days of depth — while shared plenary sessions keep SIC 2027 feeling like one conclave rather than three co-located meetings.

HALL A

Surgery & Sports Orthopaedics

Orthopaedic & arthroscopy surgeons, sports physicians, PG trainees · ~45% of delegates

HALL B

Rehabilitation, Performance & Return to Play

Sports physiotherapists, S&C coaches, athletic trainers · ~35%

HALL C

Sports Science, Nutrition & Athlete Health

Nutritionists, physiologists, team doctors, psychologists · ~20%

Room sizing: Hall A doubles as the plenary hall and must seat the full delegate count. Halls B and C need roughly 40% and 25% of that. Hall B additionally needs clear floor space for hands-on workshops.

JOINT PLENARY — EVERYONE IN ONE ROOM

These six sessions are deliberately **not** parallel. They are what make it one conclave:

SESSION	WHEN	WHY IT MUST BE JOINT
Inauguration	Day 1, 09:00	Ceremonial; sets the inaugural-edition tone
Opening keynote	Day 1, 09:15	One marquee name, heard by every delegate
The Return-to-Play Panel surgeon · physio · S&C · nutritionist · athlete	Day 1, 12:45	The signature session. The one decision every profession in the building shares — and the clearest expression of why this conclave exists
Medical ethics & good clinical practice	Day 1, 17:30	Mandatory for TSMC CPD accreditation — the Declaration Proforma requires it. Joint scheduling guarantees attendance capture
Athlete fireside chat	Day 2, 12:35	Draws every audience; strongest media moment
Awards & valedictory	Day 2, 17:00	Best paper and poster across all three tracks

The research produced roughly three days of strong content for a two-day event. The committee's first decision should be whether to add a pre-conference day on 28 January for hands-on workshops and the reserve blocks — because that decision determines the venue contract.

DAY 1 — FRIDAY 29 JANUARY 2027

TIME	HALL A — SURGERY	HALL B — REHAB & PERFORMANCE	HALL C — SCIENCE & NUTRITION
08:00	Registration · breakfast · trade exhibition		
09:00	JOINT — Inauguration		
09:15	JOINT — Opening keynote		
10:00	A1 · Instability: beyond the Bankart	B1 · Criteria-based rehab & the RTP decision	C1 · Energy availability & REDs
11:15	Tea · exhibition		
11:35	A2 · Rotator cuff: repair, augment, transfer	B2 · Objective testing, screening & technology	C2 · Nutrition periodisation on Indian diets
12:45	JOINT — THE RETURN-TO-PLAY PANEL · surgeon · physio · S&C · nutritionist · athlete		
13:30	Lunch · exhibition		
14:30	A4 · ACL I — primary reconstruction debates	B3 · Tendinopathy & load-driven pain	C3 · Hydration & heat in Indian conditions
16:00	Tea		
16:15	A5 · ACL II — complex & multiligament knee	W1 · Hands-on carousel (ticketed, capped)	C4 · Supplements & ergogenic aids
17:30	JOINT — Medical ethics & GCP in sports medicine (<i>TSMC CPD requirement</i>)		
18:15	Free papers — parallel by track		
19:30	Faculty dinner		

DAY 2 — SATURDAY 30 JANUARY 2027

TIME	HALL A — SURGERY	HALL B — REHAB & PERFORMANCE	HALL C — SCIENCE & NUTRITION
08:30	A6 · Save the meniscus	W2 · Field-based RTP testing & hamstring (hands-on)	C8 · Exercise physiology & performance testing
09:50	A7 · Cartilage & joint preservation	B5 · Muscle injury & the hamstring	C9 · Wearables, AI & athlete monitoring
11:05	Tea · exhibition		
11:25	A8 · Hip & groin	B6 · Load management & injury risk	C12 · Concussion — consensus-based practice
12:35	JOINT — Athlete fireside chat		
13:10	Lunch · exhibition		
14:10	A9 · Foot & ankle in sport	B7 · The athlete who isn't a 25-year-old male	C13 · Anti-doping & WADA 2026 updates
15:10	A10 · Paediatric & adolescent athlete	Free papers — rapid fire	C14 · Sports cardiology & screening
16:00	Tea		
16:15	A11 · Orthobiologics: evidence vs marketing	joins Hall A	C15 · Para-athlete health & surveillance

Reserve blocks (add with a pre-conference day, or swap in as faculty confirm): A3 Elbow & the overhead athlete · A12 Revision & failed surgery · B4 Adjuncts under scrutiny · C5 Gut health · C6 Female athlete health · C7 Youth athlete · C10 Recovery & sleep · C11 Sports psychology.

SURGERY & SPORTS ORTHOPAEDICS

Format: 12-minute talks (10 speak + 2 changeover), hard stops enforced by chairs. Debates 2 × 8 min + rebuttal, with audience polling before and after. **Pre-record all "live" surgery** — the operating surgeon narrates the recording live. This gives the teaching value without OT-link risk. Chair pairs, one senior + one mid-career who runs the clock.

A1 · Instability: beyond the Bankart

Measuring bone loss that changes the plan (3D-CT vs glenoid track vs MRI) · Bankart + remplissage into subcritical bone loss · Latarjet 2027: classic vs congruent-arc, screw vs button, open vs arthroscopic · free bone block after failed Latarjet · posterior and multidirectional instability in throwers and gymnasts · **relive:** arthroscopic Latarjet — the six steps where it goes wrong.

DEBATE · 19-year-old kabaddi player, first dislocation — stabilise now vs rehab and wait

A2 · Rotator cuff: repair, augment, transfer

Massive but reparable — margin convergence, interval slides, knowing when to stop pulling · SCR vs lower trapezius transfer, with fatty infiltration as the discriminator · patch and bioinductive augmentation: what 2025 evidence supports and what it doesn't · the subscapularis tear you're still missing · case panel: three cuffs, three plans.

A4 · ACL I — the primary reconstruction debates

Graft selection 2027: is quadriceps tendon now the default? (international survey — QT preferred for 61% of primaries, 89% of revisions) · hamstring sizing and fixation when the graft measures 7 mm · ACL repair with internal bracing: which 5% of tears? · ramp lesions and why they're missed at index surgery · **relive:** QT ACLR + modified Lemaire LET, narrated end to end.

DEBATE · Every high-risk primary ACL needs a lateral extra-articular tenodesis

A5 · ACL II — the complex & multiligament knee

Posterior tibial slope: when to measure, when to cut — slope-reducing osteotomy in the re-ruptured knee · the multiligament knee in India: road-traffic vs kabaddi patterns, single-stage vs staged · PCL and the residual sag nobody reports · posterolateral corner reconstruction that fits a 90-minute list · RTS criteria with no isokinetic dynamometer.

A6 · Save the meniscus

Posterior root tears — fixation angle, tension, and why extrusion persists · meniscal centralisation: promising or over-sold? · radial and horizontal cleavage tears in the young adult · "meniscectomy is not a crime" — the 15% where resection is right · allograft and scaffold pathways in India · **relive + panel:** root and ramp repair in the same knee, including the complication.

A7 · Cartilage & joint preservation

The 2 cm² decision: microfracture vs AMIC vs minced vs MACI, with Indian cost reality · single-stage minced cartilage and the 2025 matched-pair data · osteochondral auto- and allograft, and the supply problem nobody discusses · unloading osteotomy *is* treatment · RTS after cartilage surgery — 9 months is a floor, not a target · **meet the expert:** audience-submitted hardest case.

A8 · Hip & groin

Groin pain algorithm for footballers and kabaddi players that survives contact · FAI syndrome after the physiotherapy trials — who still gets a scope? · labral repair vs reconstruction vs augmentation, and does the capsule have to be closed? · **how I do it:** hip arthroscopy setup, portals, avoiding cam under-resection.

DEBATE · Borderline dysplasia (LCEA 20–25°) — arthroscopy + plication vs PAO

A9 · Foot & ankle in sport

Chronic lateral instability decision tree: arthroscopic Broström, InternalBrace, or anatomic reconstruction · osteochondral lesions of the talus — size thresholds · Achilles in the athlete · syndesmosis: button vs screw and how much malreduction matters · posterior impingement and FHL in dancers.

A10 · The paediatric & adolescent athlete

ACL in the skeletally immature — transphyseal vs all-epiphyseal vs IT-band (growth disturbance <2%, re-tear 9–15%) · tibial spine and patellar sleeve avulsions · adolescent patellar instability: MPFL alone, or must you address trochlea and TT-TG? · OCD of knee and capitellum · the overtrained 14-year-old and the academy culture problem.

A11 · Orthobiologics: evidence vs marketing

What ESSKA-ORBIT supports (PRP in KL 1–3 knee OA, lateral epicondylitis) and what it does not · BMAC, adipose and "stem cell" clinics against placebo-controlled data, plus the Indian regulatory position · consent, cost and honest claims with an Indian patient.

DEBATE · Biologics have no place in the arthroscopic OR in 2027

A12 · When it goes wrong — revision & failed surgery RESERVE

A workup that finds the real cause of the failed ACL (tunnel, slope, meniscus, alignment, rehab) · one- vs two-stage revision when autograft is spent · recurrent instability after Bankart or Latarjet · septic arthritis after arthroscopy — retain or remove the graft? · **panel:** "My worst complication" — four faculty, unedited. *Programme this last on Day 2 — it is the session people stay for.*

Sources: IASCON 2025 & 2026 programmes · ISAKOS 2025 Congress · AOSSM 2025 Annual Meeting · ESSKA 2026 · ISKSAA · LET international consensus (Arthroscopy 2025) · quadriceps tendon global survey (JEO 2026) · ramp/root/radial current concepts (JASSM 2025) · minced cartilage vs ACI (KSSTA) · ESSKA-ORBIT PRP consensus · PAO vs hip arthroscopy meta-analysis 2025 · all-epiphyseal ACLR meta-analysis 2025 · kabaddi knee injury epidemiology (Asian J Sports Med).

REHABILITATION, PERFORMANCE & RETURN TO PLAY

Format: 12–15 minute talks; blocks close with a moderated discussion rather than per-talk Q&A. Two ticketed, capped hands-on workshops — these are the reason physiotherapists pay to attend rather than watch online. **Recruit S&C faculty, not only physiotherapists:** an all-physio track loses the S&C audience by mid-morning of Day 1.

B1 · Criteria-based rehabilitation & the RTP decision

"Time is not a criterion" — what criteria-based actually means when you write the discharge note · what proportion of your athletes would truly pass a full test battery at 9 months post-ACLR · return to participation → sport → performance · psychological readiness (ACL-RSI) and the athlete whose knee passes but whose head doesn't · who owns the decision, and how it is documented. Closes by building a one-page RTP criteria sheet live from the floor.

DEBATE · No athlete should be cleared for pivoting sport before 9 months post-ACLR

B2 · Objective testing, screening & technology

Force plates in a 400-sq-ft clinic: the five countermovement-jump metrics worth reporting and the twenty that aren't · handheld dynamometry done properly — belt fixation, normalisation, minimum detectable change · isokinetics vs field testing in 2027 · movement screening after FMS: what survived the validity literature · markerless motion capture — good enough to change a clinical decision? · **panel:** "What would you buy first with ₹5 lakh?"

B3 · Tendinopathy & load-driven pain

Tendon pain as a loading problem, in one slide · isometrics for analgesia: where "45 seconds × 5" came from and how often it fails · heavy slow resistance vs eccentrics for patellar tendinopathy — the network meta-analysis verdict · insertional vs mid-portion Achilles: why the same programme fails one · gluteal tendinopathy — compression is the enemy, stretching is the trap · energy storage and release, the phase everyone skips.

W1 · Hands-on carousel TICKETED · CAPPED ~30/STATION

Station 1 — Objective testing bench: live CMJ and drop jump on volunteers, reading the force-time curve on screen; dynamometry with vs without belt fixation, showing the tester-strength artefact; writing the actual report line a surgeon and a coach will each read.

Station 2 — Tendon loading lab: Spanish squat rigged with a belt and pole, no proprietary kit; HSR tempo work; modifying every exercise for a clinic with one bench, one step and one barbell; the 24-hour pain-response rule on case vignettes.

Station 3 — Plyometric progression: pogo → hop → bound ladder with landing-quality criteria; take a 3-month post-ACLR athlete to month 7 using only cones, a box and floor space.

W2 · Field-based RTP testing & hamstring reconditioning HANDS-ON

Build a 40-minute RTP battery with no lab — 505 change of direction, hop series with a tape measure, Askling H-test, sprint exposure with timing gates, and honest caveats on phone-app timing · hamstring L-protocol and Nordic technique coached hands-on, with regressions · sprint reintroduction dosing and the criterion for the first max-effort sprint · the pass/fail sheet you hand the coach.

B5 · Muscle injury & the hamstring

Classifying muscle injury (BAMIC, MLG-R) — and whether the MRI changes what you do on Monday · L-protocol vs conventional strengthening: 28 days vs 51 days, and what newer reviews say about reinjury · lengthened-state training and the Nordic: fascicle length vs adherence reality · calf and adductor — the two you are under-rehabbing · from return to running to return to sprinting.

B6 · Load management & injury risk

What happened to ACWR — from "predicts injury" to a cluster RCT in 482 elite youth footballers showing no difference in injury rates · metrics that survive a real Indian club season: sRPE, weekly change, chronic capacity, low-tech monitoring that actually gets collected · bowling workload and the fast bowler's back · sleep, energy availability and travel — the load that never appears in the GPS file.

B7 · The athlete who isn't a 25-year-old male

Youth resistance training is protective — the position-statement evidence, and how to say it to a parent and a coach · the maturing athlete: apophyseal injury, load modification not rest · the female athlete and the menstrual cycle: what genuinely low-quality evidence permits you to claim · REDs as a modifiable rehabilitation variable · para-athlete rehabilitation and why your normative data doesn't apply.

B4 · Adjuncts under scrutiny RESERVE

Blood flow restriction: what 2024–25 meta-analyses actually show (comparable, not superior) and the three groups where the cuff earns its place · manual therapy as a short-half-life adjunct — using it without building dependence · shockwave, dry needling, and the physiotherapist's filter for injectable-adjacent claims.

Sources: RTS tests and criteria after ACLR scoping review 2025 · AOSSM RTP metrics (Winter 2025) · LSI overestimates knee function (JOSPT) · BFR vs general rehab after ACLR (KSSTA 2025) · eccentric vs isometric vs HSR network meta-analysis · hamstring rehab comparative effectiveness · ACWR systematic review and meta-analysis · FMS predictive validity · NSCA youth resistance training position statement · IFSPT / World Physiotherapy Congress 2025 · kabaddi knee injury epidemiology.

SPORTS SCIENCE, NUTRITION & ATHLETE HEALTH

Format: 12-minute talks, four per hour plus a chaired panel. Blocks marked **CONSENSUS** rest on a formal consensus document — cite it on the slide; it strengthens the CME file. Blocks marked **INDIA** carry an India-specific angle. **REDs and concussion need physician co-chairs** — both carry diagnosis and return-to-play liability.

C1 · Energy availability & REDs **CONSENSUS** **INDIA**

From RED-S to REDs: what actually changed in the 2023 IOC consensus, and what it changes on Monday morning · adaptable vs problematic low energy availability — applying the CAT2 traffic-light stratification across a 30-athlete squad · screening for low energy availability without a DXA in an Indian academy · male REDs, the missed diagnosis: weight-making in wrestling, boxing and weightlifting.

C2 · Nutrition periodisation on Indian diets **INDIA**

Fuel for the work required — periodising carbohydrate across a 12-week block without inducing low energy availability · hitting 1.6–2.2 g/kg protein on a vegetarian Indian diet: dal, dairy and the leucine threshold problem · building a pre-competition menu from rice, roti, poha and idli rather than translating a Western one · feeding the residential hostel athlete: what a SAI or Khelo India mess can and cannot deliver.

C3 · Hydration & thermoregulation in Indian heat **CONSENSUS** **INDIA**

WBGT is not air temperature — writing a heat policy an Indian match referee will actually enforce · sweat rate and sweat sodium at 33°C and 85% humidity, and why imported hydration prescriptions under-deliver · exertional heat stroke on the field: cool first, transport second — cold-water immersion below 39°C *before* the ambulance moves · ten-day heat acclimatisation for travelling teams.

C4 · Supplements & ergogenic aids **CONSENSUS** **INDIA**

The five that survive scrutiny — creatine, caffeine, beta-alanine, nitrate, bicarbonate: effect sizes, not enthusiasm · contamination risk in the Indian supplement market and what third-party batch certification does and doesn't cover · strict liability explained to an athlete · ayurvedic and herbal ergogenics: ashwagandha, shilajit, and the evidence-to-marketing gap.

C8 · Exercise physiology & performance testing

Durability — the endurance parameter your VO₂max test misses · critical power and critical speed as field tools: three tests on a track in one session · lactate thresholds without a lab, and the errors that change prescription · force plates and the countermovement jump: which metrics track fatigue and which track noise.

C9 · Wearables, AI & athlete monitoring **INDIA**

Only ~43% of wearable studies report adequate validity and reliability — a buyer's checklist before your federation signs · GPS and workload after a decade of criticism: what survives · why AI injury-prediction models keep failing external validation, and what to demand from a vendor · **athlete data governance under India's DPDP Act**: consent, ownership and the federation contract.

C12 · Concussion **CONSENSUS** **INDIA**

Amsterdam 2022 in practice: SCAT6, SCOAT6 and the 10–15 minute multimodal screen the sideline never gives you · the new visible signs mandating immediate removal — tonic posturing, impact seizure, ataxia, blank look · Child SCAT6 and school sport: a protocol an Indian school can run without a physician on site.

C13 · Anti-doping & WADA 2026 updates **CONSENSUS** **INDIA**

The 2026 Prohibited List decoded for team clinicians — carbon monoxide as new M1.4, cell components, revised salmeterol intervals · building a TUE file that gets approved · **India top of WADA's violation table three years running**: 7,113 tests, 260 adverse findings, 3.6% positivity — read honestly · where India's positives cluster (athletics 76, weightlifting 43, wrestling 29) and what support staff can actually change.

Recommendation: record this block and publish it free. Federation support staff who never register are exactly the audience that needs it.

C14 · Sports cardiology & pre-participation screening CONSENSUS INDIA

What is normal in a trained heart — the International Criteria and the 2–7% abnormal-ECG downstream burden · Asia-Pacific Society of Cardiology screening recommendations and their fit to South Asian athletes · screening at scale in Indian school and academy sport: what is deliverable, and what to do when it isn't.

C15 · Para-athlete health & injury surveillance CONSENSUS INDIA

Thermoregulation in para-athletes — spinal cord injury, impaired sudomotor function, autonomic dysreflexia, and Indian heat · nutrition for the wheelchair athlete · the IOC 2020 surveillance consensus and STROBE-SIIS: what it would take to build a national injury registry for Indian sport · **closing panel:** what Hall C should be able to measure by SIC 2029.

Reserve blocks: C5 gut health & fuelling tolerance · C6 female athlete health & the menstrual cycle · C7 youth & academy athlete (bio-banding, age fraud, minimum-viable screening) · C10 recovery, sleep & travel · C11 sports psychology & mental health.

Sources: IOC 2023 REDs consensus (BJSM 57:1073–1097) · REDs CAT2 validation · WADA 2026 Prohibited List and Summary of Modifications · WADA 2024 testing figures · Amsterdam 2022 concussion consensus · ACSM expert consensus on exertional heat illness · 2025 Female Athlete Triad Coalition update · IOC 2020 injury and illness surveillance consensus (STROBE-SIIS) · IOC 2019 mental health consensus · supplement contamination systematic review · APSC pre-participation screening recommendations · India DPDP Act 2023.

THE THREADS THAT MAKE IT OURS

A translated Western programme is the failure mode of a new conference. These threads run through all three halls and give SIC 2027 an identity no international meeting can copy.

- **Kabaddi.** Indian series data show ACL tears in 89.5% of injured players, 68.4% with meniscal involvement, **57.9% never returning to the sport**, and 16.6 months mean time lost among those who do. That single dataset earns a case in the ACL, meniscus, hip/groin and rehabilitation sessions.
- **Cricket bowling workload** and the fast bowler's back — across surgical, rehabilitation and load-management halls.
- **Heat and humidity.** Heat policy, sweat sodium, acclimatisation. Non-negotiable for an Indian audience and absent from most imported programmes.
- **Vegetarian athlete nutrition** — protein targets and the leucine threshold on Indian diets.
- **Doping.** India has led WADA's violation table three years running. Confronting it directly is the most valuable thing Hall C can do for Indian sport.
- **Rehabilitation without a lab.** Most Indian clinics have no isokinetic dynamometer or force plate. Every rehab talk should state what to do without one.
- **Late presentation.** Kabaddi ACL injuries present at a mean of ~14 months. Programme it as a clinical reality, not a footnote.

WHAT WE NEED SIGNED OFF

1 · Two days, or two plus a pre-conference day?

The research yields roughly three days of strong content. A 28 January pre-conference day would hold the hands-on workshops, any dry-lab work and the reserve blocks. **Take this decision first — it determines the venue contract.**

2 · Confirm the three-hall split and approve the hall titles.

Hall A doubles as the plenary hall and must seat the full delegate count.

3 · Faculty scale.

Roughly **75–95 speaking slots** across three halls over two days, plus chairs — about 40–50 faculty at two talks each. For an inaugural edition, lock 2–3 marquee anchors first and recruit everyone else off their names. Confirm in writing 6–9 months out and keep a reserve bench.

4 · Which debates run.

Two per hall per day is the ceiling. More and they stop feeling like events.

5 · The ethics / GCP session stays as a joint plenary.

TSMC's Declaration Proforma requires it for CPD accreditation. Non-negotiable.

6 · Workshop ticketing.

Hands-on sessions must be capped and ticketed at registration with per-station rosters. Uncapped "hands-on" becomes a lecture with mats on the floor.

7 • Equipment sponsorship boundary.

Force plates and dynamometers will be vendor-supplied — tie them to an exhibitor package, but keep teaching faculty independent of the vendor and say so from the podium. This is the single biggest credibility risk in the rehabilitation track.

8 • Free-paper prizes per track.

So physiotherapy and nutrition entries don't compete against surgical case series.

Two research caveats, stated plainly. The ISAKOS, AOSSM and ESSKA programme pages blocked direct retrieval, so Hall A structure is reconstructed from search-indexed programme and abstract listings rather than read off the printed programme — worth checking the ESSKA 2026 highlight-lecture list before finalising. And IASMCN does not publish a session-level programme in machine-readable form, so Hall C is built from the consensus literature and Indian data rather than a competitor programme — a call to the IASM secretariat before finalising would de-risk duplication.